A Phase II, Randomized, Double■Blind, Placebo■Controlled Study

Protocol Number: PROTO■001

NCT Number: NCT12345678

Principal Investigator: Dr. Ananya R. Mehta, MD, PhD

Sponsor: BioAdvance Therapeutics Pvt. Ltd.

Funded by: National Institutes of Health (NIH)

Version Number: v1.0

Date: 2026-02-22

TABLE OF CONTENTS

• STATEMENT OF COMPLIANCE
• PROTOCOL APPROVAL & AGREEMENT
• 1 PROTOCOL SUMMARY
• 2 INTRODUCTION
• 3 OBJECTIVES AND ENDPOINTS
• 4 STUDY DESIGN
• 5 STUDY POPULATION
• 6 STUDY INTERVENTION
• 7 STUDY INTERVENTION DISCONTINUATION AND PARTICIPANT DISCONTINUATION/WITHDRAWAL
• 8 STUDY ASSESSMENTS AND PROCEDURES
• 9 STATISTICAL CONSIDERATIONS
• 10 SUPPORTING DOCUMENTATION AND OPERATIONAL CONSIDERATIONS
• 11 REFERENCES
• APPENDICES

STATEMENT OF COMPLIANCE

The trial will be carried out in accordance with International Conference on Harmonisation Good Clinical Practice (ICH GCP)...

PROTOCOL APPROVAL & AGREEMENT

Protocol Name:	A Phase II, Randomized, Double■Blind Study
Protocol Number:	PROTO■001
IMP:	XYZ■101 (Inhalation Suspension)
Indication:	Moderate-to-Severe Asthma
Clinical Phase:	phase-II
Investigators:	Dr. Ananya R. Mehta, MD, PhD – Principal Investigator
Coordinating Investigator:	Dr. Ananya R. Mehta, MD, PhD
Expert Committee:	Dr. N. Raghavan, Clinical Pharmacology Expert Dr. Latha Venugopal, Pulmonology

GCP Statement

This clinical trial will be conducted in full compliance with the principles outlined in the International Council for Harmonisation (ICH) Guideline for Good Clinical Practice (E6 R2/R3), the Declaration of Helsinki, and applicable local regulatory requirements. All investigators agree to adhere to these guidelines to ensure the rights, safety, and well■being of trial participants.

Approval Statement

By signing this document, the undersigned acknowledge approval of this clinical trial protocol and agree to conduct the study in accordance with its provisions, the Sponsor's standard operating procedures (SOPs), applicable regulatory requirements, and ICH■GCP guidelines. No deviations or modifications to the protocol may be implemented without written approval from the Sponsor and Ethics Committee, except where necessary to eliminate immediate hazards to participants.

Sponsor Representatives

Name: Mr. R. Sandeep Kumar

Title: Director, Clinical Development

Organization:

Date: 22/2/2026

Name: Dr. Latha Venugopal

Title: Pulmonology Specialist

Organization:

Date: 22/2/2026

Investigator Agreement

I, the undersigned Investigator, confirm that I have read and understood this clinical trial protocol (Version 1.0 dated 22■02■2026). I agree to conduct this study in accordance with the protocol, ICH■GCP guidelines, applicable ethical and regulatory requirements, and to

ensure the protection, safety, and confidentiality of all study participants.

Investigator Name: Dr. Ananya R. Mehta

Title: Principal Investigator, Pulmonary Medicine

Facility: Chennai Clinical Research Institute

Date: 22-02-2026

1 PROTOCOL SUMMARY

1.1 Synopsis

Title: A Phase II, Randomized, Double■Blind, Placebo■Controlled Study

Clinical Phase: Phase-II

Primary Objectives

• To evaluate the efficacy of XYZ■101 in reducing the frequency of asthma exacerbations.

• Especially in adults with moderate■to■severe asthma.

1.2 Schema

Table of Contents

Right-click and select 'Update Field' to refresh the Table of Contents.

Table of Contents 1
STATEMENT OF COMPLIANCE 2
1 PROTOCOL SUMMARY 3
1.3 Schedule of Activities (SoA) 4
2 INTRODUCTION 5
2.1 Study Rationale 6
2.2 Background 6
2.3 Risk/Benefit Assessment 6
2.3.1 Known Potential Risks 6
2.3.2 Known Potential Benefits 6
2.3.3 Assessment of Potential Risks and Benefits 6
3 OBJECTIVES AND ENDPOINTS 6
STUDY DESIGN 7
4.1 Subsection 8
4.2 Subsection 8
4.3 Subsection 8
4.4 Subsection 8
STUDY POPULATION 8

Table of contents

Detailed overview of the study management

	A	B	C	D	E	F	G	H	I	J	K	L	M	N	O	P	Q	R	S
1	**0**	**1**	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	**11**	**12**	**13**	**14**	**15**	**16**	**17**	**18**
2	Trial Activi	Run-in	Screening Period		Treatment Period													Follow-up Period	
3	Week				0		1		2	4	6	8	10				12		15
4	Day(s)		-28 to -6	-5 to -1	0	2	6	10	14	28	42	56	70	72	76	80	84	90	105
5	Allowed window for visit (days)						±1	±1	±1	±1	±2	±2	±2			±1	±2	±2	±2
6	Visits	Visit 0 Stal	Visit 1 Scre	Visit 1.1 B	Visit 2 Bas	V 2.1	V 2.2	V2.3	V 3	V 4	V 5	V 6	V 7	V 7.1	V 7.2	V7.3	V 8 (EoT V	V 8.1	V 9 (Safety
7	Informed (	•	•a																
8	Inclusion/exclusion cı		•		•														
9	Stable con	•	•	•	•				•	•	•	•	•				•		•
10	HIV, HBV, HCV, EBV,		•																
11	TB test		•																•
12	Randomisation				•														
13	Demograp	•	•																
14	Alcohol and tobacco		•																
15	Medical and surgical		•																
16	UC medice	•	•																
17	Urine screening for d		•																
18	Endoscopyb including		•c														•		
19	Diary dispensing		•	•	•				•	•	•	•	•						
20	Diary reviewd			•	•				•	•	•	•	•				•		
21	Efficacy evaluation (full Mayo score)				•												•		
22	Efficacy evaluation (9-point partial Mayo sc				•				•	•	•	•	•				•		
23	Blood sampling for PK assessmentse				•	•	•	•	•	•	•	•	•	•	•	•	•	•	•

Trial activity

Patients visit window

1.3 Schedule of Activities (SoA)

Procedures	Screening	Enrollment/Baseline	Visit 1	Visit 2	Visit 3	Visit 4	Visit 5	Visit 6	Visit 7	Visit 8	Visit 9	Visit 10	Visit 11	Visit 12	Final Visit
Informed consent	X	X	X												
Demographics						X			X		X		X		X
Medical history			X		X		X			X				X	X
Randomization		X			X		X	X	X		X		X		X
Administer study intervention		X											X		X
Concomitant medication review	X		X		X		X	X		X			X		
Physical exam		X								X	X		X	X	
Vital signs												X		X	
Height			X			X			X		X		X		X
Weight															X
Performance status											X		X		
Hematology	X		X				X		X		X		X		
Serum chemistry															
Pregnancy test		X	X		X		X	X		X			X		X
EKG															X
Adverse event review							X			X					
Radiologic/Imaging	X		X			X						X			
Other assessments										X			X		X
Complete CRFs		X			X						X				

2 INTRODUCTION

Asthma is a chronic inflammatory disorder of the airways characterized by recurrent episodes of wheezing, breathlessness, chest tightness, and coughing. Despite the availability of inhaled corticosteroids (ICS), long■acting β■■agonists (LABA), and biologic therapies, a significant proportion of patients continue to experience uncontrolled asthma, frequent exacerbations, and reduced quality of life. This unmet need has driven the development of novel therapeutic agents aimed at targeting underlying airway inflammation through more precise mechanisms. XYZ■101 is an investigational inhalation therapy developed to modulate inflammatory pathways implicated in moderate■to■severe asthma. Preclinical studies have demonstrated its potential to reduce airway hyper■responsiveness, decrease eosinophilic inflammation, and improve lung function parameters. Early■phase clinical data have shown acceptable safety and tolerability, supporting advancement to Phase II evaluation. This clinical trial has been designed to further investigate the safety, tolerability, and efficacy of XYZ■101 compared with placebo in adult participants living with moderate■to■severe asthma. The study includes a robust, randomized, double■blind, placebo■controlled design that aligns with international regulatory and scientific standards. The results from this trial will provide critical data to guide further clinical development and potential progression to Phase III confirmatory trials. The introduction also outlines the scientific rationale, disease burden, therapeutic gaps, and the developmental context for XYZ■101, forming the basis for the study objectives and methodology described in subsequent sections of this protocol.

The title should be easy to remember, recognizable by administrative support staff, and sufficiently different from other protocol titles to avoid confusion. Brevity with specificity and neutrality is the goal. If there is a "short title" (e.g., an abbreviation used to refer to the study title, include here and that can be used throughout this document in place of the full title).

Principal Investigator:

Sponsor means an individual or pharmaceutical or medical device company, governmental agency, academic institution, private organization, or other organization who takes responsibility for and initiates a clinical investigation.

All versions should have a version number and a date. Use the international date format (day month year) and write out the month (e.g., 23 June 2015).

Summary of Changes from Previous Version:

Affected Section(s)	Summary of Revisions Made	Rationale

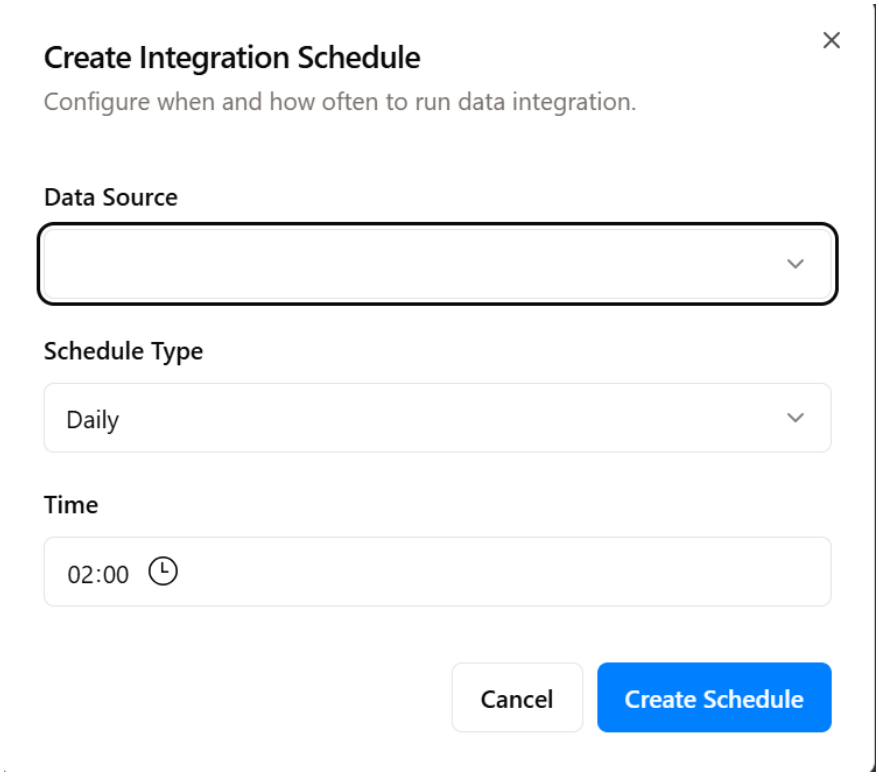
Principal investigator

investigates related to the clinical practicies

2.1 Study Rationale

Although the sample size was calculated considering only Cw6 haplotype, it will also be adequate for a regression model assessing differences from baseline of clinically relevant

variables such as BMI, metabolic syndrome, smoking and TNFα-308 polymorphism, which, among the six existing polymorphisms, is the one most greatly affecting response to treatment with anti-TNFα drugs.



Integration
Schedules the time

2.2 Background

The purpose of this amendment is to provide continued treatment of patients on secukinumab for an additional 48 weeks (overall up to 72 weeks), and thus allow for safety, tolerability, and efficacy data to be collected from the participating patients over a longer period of time.

2.3 Risk/Benefit Assessment

Furthermore, as a consequence of the European Medicines Agency's (EMA) approval in February 2015 of secukinumab "for the treatment of moderate severe plaque psoriasis in adults who are candidates for systemic therapy," inclusion criterion number 5 has been changed in order to guarantee treatment to the population of systemically naïve patients, as per label. The study extension can be terminated by Novartis for any reason after a period of at least 12 weeks. Termination may occur only in selected regions where the drug is commercially available; this decision will be made on a region-by-region basis, as for some regions the study will continue as planned even though secukinumab may have become commercially available.

Table 1 Time and Events Schedule

Trial Activity	Run-in	Screening Period		Treatment Period									
Week				0		1		2	4	6	8	10	
Day(s)		-28 to -6	-5 to -1	0	*2*	*6*	*10*	14	28	42	56	70	72
Allowed window for visit (days)						*±1*	*±1*	±1	±1	±2	±2	±2	
Visits	**Visit 0 Stabilization**	**Visit 1 Screening**	**Visit 1.1 Baseline**	**Visit 2 Baseline**	***V 2.1***	***V 2.2***	***V2.3***	**V 3**	**V 4**	**V 5**	**V 6**	**V 7**	***V 7.1***
Informed consent	•	•[a]											
Inclusion/exclusion criteria		•		•									
Stable conventional treatment	•	•	•	•				•	•	•	•	•	
HIV, HBV, HCV, EBV, CMV test		•											
TB test		•											
Randomisation				•									
Demographics	•	•											
Alcohol and tobacco habits		•											
Medical and surgical history		•											
UC medical history, confirmation of UC diagnosis, and previous UC therapy	•	•											
Urine screening for drugs of abuse		•											
Endoscopy[b] including biopsy sampling		•[c]											

Times and events

Patients visit window times

2.4 Known Potential Risks

Title: “A 24-week, multicenter, proSpective stUdy to evaluate the PASI 90 clinical response rate and the safety PRofile of sEcukinuMab 300 mg in Cw6negativE and Cw6-positive patients with moderate to severe chronic plaque-type psoriasis (SUPREME) – amended with an extension treatment period of up to 48 weeks”

2.5 Known Potential Benefits

Although the sample size was calculated considering only Cw6 haplotype, it will also be adequate for a regression model assessing differences from baseline of clinically relevant variables such as BMI, metabolic syndrome, smoking and TNFα-308 polymorphism, which, among the six existing polymorphisms, is the one most greatly affecting response to treatment with anti-TNFα drugs.

2.6 Assessment of Potential Risks and Benefits

Changes to specific sections of the protocol are shown in the track changes version of the protocol using strike through red font for deletions and red underlined font for insertions.

3 OBJECTIVES AND ENDPOINTS

4 STUDY DESIGN

If a prohibited treatment listed in Table 5-1 is used during the study, the patient must discontinue use of the prohibited treatment if he/she wishes to continue in the study. At the discretion of the investigator, if the patient’s use of a prohibited treatment presents undue safety risk for the patient, he/she should be discontinued from study treatment as per Section 5.5.9.

4.1 Overall Design

Topical treatment that is likely to impact signs and symptoms of psoriasis (e.g. vitamin D analogues, - 2 weeks - 2 weeks - pimecrolimus, - retinoids, - salicylvaseline, salicylic acid, lactic acid, tacrolimus, tar, urea alpha-hydroxy or fruit acids)

4.2 Scientific Rationale for Study Design

Patients may voluntarily withdraw from the study for any reason at any time. They may be considered withdrawn if they state an intention to withdraw, fail to return for visits, or become lost to follow-up for any other reason. Patients may voluntarily withdraw from the study for any reason at any time. They may be considered withdrawn if they state an intention to withdraw, fail to return for visits, or become lost to follow-up for any other reason. If premature withdrawal occurs for any reason, the investigator must make every effort to determine the primary reason for a patient’s premature withdrawal from the study and record this information.

4.3 Justification for Dose

The investigational treatment must be discontinued under the following circumstances: 1. Withdrawal of informed consent 2. Severe adverse events that, in the opinion of the Investigator, warrant discontinuation 3. Pregnancy 4. Use of prohibited treatment as per Table 5-1 Any other protocol deviation that results in a significant risk to the patient’s safety

4.4 End of Study Definition

Patients that do not achieve PASI 50 at week 16 will terminate treatment. For patients who are lost to follow-up (i.e. those patients whose status is unclear because they fail to appear for study visits without stating an intention to withdraw), the investigator should show "due diligence" by documenting the steps taken to contact the patient, e.g. dates of telephone calls, registered letters in the source, etc. Patients who are prematurely withdrawn from the study will not be replaced by an equal number of newly enrolled patients.

5 STUDY POPULATION

The core study will have a duration of 24 weeks. Following the evaluation of psoriasis severity at week 16, patients achieving PASI 50 response will be eligible to continue on the study treatment for an additional 8 weeks. Willing patients reaching at least a PASI 75 response during the core study will be eligible to enter an extension treatment period of an additional 48 weeks. The investigator must provide follow-up medical care for all patients who are prematurely withdrawn from the study, or must refer them for appropriate ongoing care.

5.1 Inclusion Criteria

The study can be terminated at any time for any reason by Novartis. Should this be necessary, the patient should be seen as soon as possible and treated as a prematurely withdrawn patient. The investigator may be informed of additional procedures to be followed in order to ensure that adequate consideration is given to protect the patient’s interests. The investigator will be responsible for informing Ethics Committees (ECs) of the early termination of the trial. The study extension can be terminated by Novartis for any reason after a period of at least 12 weeks. Termination may occur only in selected regions where the drug is commercially available; this decision will be made on a region-by-region basis, as for some regions the study will continue as planned even though secukinumab may have become commercially available.

5.2 Exclusion Criteria

Investigational treatment must be received by a designated person at the study site, handled and stored safely and properly, and kept in a secured location to which only the investigator and designees have access. Upon receipt, all investigational treatment should be stored according to the instructions specified on the labels. Clinical supplies are to be dispensed only in accordance with the protocol.

5.3 Lifestyle Considerations

Medication labels will be in the local language and comply with the legal requirements of each country. The investigator must maintain an accurate record of the shipment and dispensing of investigational treatment in a drug accountability log. Monitoring of drug accountability will be performed by the field monitor during site visits and at the completion of the trial.

5.4 Screen Failures

At the conclusion of the study, and as appropriate during the course of the study, the investigator will return all unused investigational treatment, packaging, drug labels, and a copy of the completed drug accountability log to the Novartis monitor or to the Novartis address provided in the investigator folder at each site. Handling of other study treatment: not applicable.

5.5 Strategies for Recruitment and Retention

At the conclusion of the study, and as appropriate during the course of the study, the investigator will return all unused investigational treatment, packaging, drug labels, and a copy of the completed drug accountability log to the Novartis monitor or to the Novartis address provided in the investigator folder at each site. Handling of other study treatment: not applicable.

6 STUDY INTERVENTION

All patients will receive Secukinumab 300 mg subcutaneously using the Phase III regimen: 300 mg (two 150 mg injections) a week for the first five weeks (visits 3-7), and once every 4 weeks thereafter (visits 8-12). Patients proceeding to the extension period will self-administer treatment (two 150 mg injections) once every 4 weeks for an additional 48 weeks. The study treatment solution must be injected subcutaneously in non-affected areas of the skin. Following the evaluation of psoriasis severity at 16 weeks, patients achieving a PASI 50 response will be eligible to continue on the study treatment for another 8 weeks. Their PASI score will be evaluated at 24 weeks. Willing patients reaching at least a PASI 75 response at the end of the 24-week core study will enter an extension treatment period of an additional 48 weeks. All dates of injections given to the subject during the study must be recorded on the Dosage Administration Record e-CRF.

6.1 Study Intervention(s) Administration

The investigator may prescribe any medications and/or supportive care during the study based on clinical needs. Use of rescue medication must be recorded on the Concomitant medications/Significant non-drug therapies/Dose Administration Record in the Electronic Case Report Form (eCRF).

6.2 Study Intervention Description

Investigational treatment must be received by a designated person at the study site, handled and stored safely and properly, and kept in a secured location to which only the investigator and designees have access. Upon receipt, all investigational treatment should be stored according to the instructions specified on the labels. Clinical supplies are to be dispensed only in accordance with the protocol.

6.3 Dosing and Administration

The investigator should instruct the patient to notify the study site about any new medications he/she takes after being enrolled into the study. All medications, procedures and significant non-drug therapies (including physical therapy and blood transfusions) administered after the patient was enrolled into the study must be recorded.

6.4 Preparation/Handling/Storage/Accountability

Patients may voluntarily withdraw from the study for any reason at any time. They may be considered withdrawn if they state an intention to withdraw, fail to return for visits, or become lost to follow-up for any other reason. If premature withdrawal occurs for any reason, the investigator must make every effort to determine the primary reason for a patient’s premature withdrawal from the study and record this information.

6.5 Acquisition and accountability

The investigator should discontinue the investigational treatment for a given patient and/or withdraw the patient from the study if, on balance, he/she believes that continuation would be detrimental to the patient’s well-being.

6.6 Formulation, Appearance, Packaging, and Labeling

The investigational treatment must be discontinued under the following circumstances: 1) Withdrawal of informed consent 2) Severe adverse events that, in the opinion of the Investigator, warrant discontinuation 3) Pregnancy

6.7 Product Storage and Stability

Patients that do not achieve PASI 50 at week 16 will terminate treatment. For patients who are lost to follow-up (i.e. those patients whose status is unclear because they fail to appear for study visits without stating an intention to withdraw), the investigator should show "due diligence" by documenting the steps taken to contact the patient, e.g. dates of telephone calls, registered letters in the source, etc. Patients who are prematurely withdrawn from the study will not be replaced by an equal number of newly enrolled patients.

6.8 Preparation

The study can be terminated at any time for any reason by Novartis. Should this be necessary, the patient should be seen as soon as possible and treated as a prematurely withdrawn patient. The investigator may be informed of additional procedures to be followed in order to ensure that adequate consideration is given to protect the patient’s interests. The investigator will be responsible for informing Ethics Committees (ECs) of the early termination of the trial.

6.9 Measures to Minimize Bias: Randomization and Blinding

The study extension can be terminated by Novartis for any reason after a period of at least 12 weeks. Termination may occur only in selected regions where the drug is commercially

available; this decision will be made on a region-by-region basis, as for some regions the study will continue as planned even though secukinumab may have become commercially available.

6.10 Study Intervention Compliance

HLA-Cw6 determination will be performed at screening for patients who have provided written informed consent. A central laboratory will be used to determine Cw6. Patients will thereafter be stratified into the two cohorts. TNFα-308 polymorphism will be centrally determined at the same time.

6.11 Concomitant Therapy

The study can be terminated at any time for any reason by Novartis. Should this be necessary, the patient should be seen as soon as possible and treated as a prematurely withdrawn patient. The investigator may be informed of additional procedures to be followed in order to ensure that adequate consideration is given to protect the patient's interests. The investigator will be responsible for informing Ethics Committees (ECs) of the early termination of the trial. The study extension can be terminated by Novartis for any reason after a period of at least 12 weeks. Termination may occur only in selected regions where the drug is commercially available; this decision will be made on a region-by-region basis, as for some regions the study will continue as planned even though secukinumab may have become commercially available.

6.12 Rescue Medicine

The investigator will be responsible for informing Ethics Committees (ECs) of the early termination of the trial. The study extension can be terminated by Novartis for any reason after a period of at least 12 weeks. Termination may occur only in selected regions where the drug is commercially available; this decision will be made on a region-by-region basis, as for some regions the study will continue as planned even though secukinumab may have become commercially available.

7 STUDY INTERVENTION DISCONTINUATION AND PARTICIPANT DISCONTINUATION/WITHDRAWAL

7.1 Discontinuation of Study Intervention

7.2 Participant Discontinuation/Withdrawal from the Study

7.3 Lost to Follow-Up

8 STUDY ASSESSMENTS AND PROCEDURES

8.1 Efficacy Assessments

8.2 Safety and Other Assessments

8.3 Adverse Events and Serious Adverse Events

8.4 Definition of Adverse Events (AE)

8.5 Definition of Serious Adverse Events (SAE)

8.6 Classification of an Adverse Event

8.7 Time Period and Frequency for Event Assessment and Follow-Up

8.8 Adverse Event Reporting

8.9 Serious Adverse Event Reporting

8.10 Reporting Events to Participants

8.11 Events of Special Interest

8.12 Reporting of Pregnancy

8.13 Unanticipated Problems

8.14 Definition of Unanticipated Problems (UP)

8.15 Unanticipated Problem Reporting

8.16 Reporting Unanticipated Problems to Participants

9 STATISTICAL CONSIDERATIONS

9.1 Statistical Hypotheses

9.2 Sample Size Determination

9.3 Populations for Analyses

9.4 Statistical Analyses

9.5 General Approach

9.6 Analysis of the Primary Efficacy Endpoint(s)

9.7 Analysis of the Secondary Endpoint(s)

9.8 Safety Analyses

9.9 Baseline Descriptive Statistics

9.10 Planned Interim Analyses

9.11 Sub-Group Analyses

9.12 Tabulation of Individual participant Data

9.13 Exploratory Analyses

10 SUPPORTING DOCUMENTATION AND OPERATIONAL CONSIDERATIONS

10.1 Regulatory, Ethical, and Study Oversight Considerations

10.2 Informed Consent Process

10.3 Study Discontinuation and Closure

10.4 Confidentiality and Privacy

10.5 Future Use of Stored Specimens and Data

10.6 Key Roles and Study Governance

10.7 Safety Oversight

10.8 Clinical Monitoring

10.9 Quality Assurance and Quality Control

10.10 Data Handling and Record Keeping

10.11 Protocol Deviations

10.12 Publication and Data Sharing Policy

10.13 Conflict of Interest Policy

10.14 Additional Considerations

10.15 Abbreviations

10.16 Protocol Amendment History

11 REFERENCES